

Somatic Shadow Work

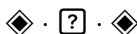
You may feel a familiar pull to intellectualize what you just mapped. To analyze, explain, or “figure out” why your jaw clenches or your belly tightens. Resist that urge. The mind is not the healer here—**the body is**.

The shadow does not live in your memories—it lives in your **muscle memory**. When trauma occurs, the mind may forget (dissociation, repression, denial), but the body **holds the score**. The fascia tightens. The vagus nerve dysregulates. The gut-brain axis signals danger long after the threat has passed.

This is why traditional talk therapy often fails to create lasting change. You can understand your childhood wounds for decades and still flinch when someone raises their voice. You can know intellectually that you are safe and still feel your heart race in conflict. **Knowing is not enough**. The shadow must be felt, moved, and released through the soma.

Polyvagal theory teaches us that the autonomic nervous system has three states: ventral vagal (safe and social), sympathetic (fight or flight), and dorsal vagal (freeze or collapse). Most shadow work happens in dorsal—you cannot think your way out of freeze. You must **breathe, move, sound, or touch** your way back to regulation.

This journal prioritizes sensation over story, embodiment over insight. You will not “understand” your shadow into submission. You will **feel** it, name it, and slowly, gently, bring it home.



Examples of where emotions live in the body:

- **Shame:** The belly, curling inward, hiding.

- **Grief:** The chest, heaviness, collapsed posture.
- **Rage:** The jaw, fists, shoulders—armored vigilance.
- **Fear:** The throat, constricted breath, shallow breathing.

As you move through the phases ahead, you will learn to translate these sensations into symbols, and symbols into release.